

Surgical Site Infection (SSI) Prevention

Policy Number	POL-IC-023	Owner / Department	Infection Prevention & Control / Perioperative Services
Effective Date	2023-10-01	Version	2.3
Last Revised	2026-03-12	Next Review	2028-03-12
Approved By	Infection Prevention & Control Committee	Applies To	All operative and invasive-procedure areas, surgeons, anesthesia, and perioperative nursing

1. Purpose

To prevent surgical site infections through evidence-based perioperative practices, consistent with the CDC Guideline for the Prevention of Surgical Site Infection (2017), the WHO Global Guidelines, and AORN perioperative standards.

2. Definitions

- **Superficial incisional SSI** — infection within 30 days involving only skin and subcutaneous tissue of the incision (NHSN).
- **Deep incisional SSI** — infection involving deep soft tissues (fascia and muscle) of the incision.
- **Organ/space SSI** — infection involving any part of the anatomy opened or manipulated during the operation, within the NHSN surveillance window (30 or 90 days by procedure).
- **Normothermia** — maintenance of perioperative core temperature at or above 36°C.

3. Perioperative Bundle

Element	Standard
Antimicrobial prophylaxis	Administer within 60 minutes before incision (within 120 minutes for vancomycin or a fluoroquinolone); redose for long cases or major blood loss; discontinue appropriately after closure.
Hair removal	Do not remove hair unless it interferes with the operation; if needed, use clippers — never a razor.
Skin antisepsis	Alcohol-based antiseptic skin prep unless contraindicated; allow to dry fully before draping.
Glycemic control	Maintain perioperative blood glucose below 200 mg/dL in patients with and without diabetes.
Normothermia	Maintain perioperative normothermia ($\geq 36^{\circ}\text{C}$).

IMPORTANT — Prophylactic antimicrobials must be fully infused within 60 minutes before incision — 120 minutes for vancomycin or a fluoroquinolone. Do not proceed to incision for an elective case until indicated prophylaxis has been given and the surgical time-out is complete.

4. Severity & Reporting

SSIs are classified under POL-RM-013. An organ/space SSI, or any SSI causing sepsis, unplanned return to the OR, or death is SEV-1 — notify the attending surgeon, Charge Nurse, Nursing Supervisor, Risk Management, Infection Prevention, and the Administrator-on-Call immediately and within 1 hour. A deep or superficial incisional SSI requiring antibiotics or bedside intervention is SEV-2, reported to the surgeon, Charge Nurse, Risk Management, and Infection Prevention within 4 hours. A bundle miss (e.g., late prophylaxis) caught before harm is a Near Miss, reported within 24 hours. A retained foreign object or wrong-site procedure is a Serious Reportable Event and always SEV-1.

5. Documentation Requirements

- Antibiotic agent, dose, and infusion time relative to incision; redosing.
- Skin-prep agent, hair-removal method, and documented perioperative temperature and glucose values.
- For a confirmed SSI: depth classification, organism, procedure and surveillance window, assigned severity, and all notifications; reported to NHSN by Infection Prevention.

6. References

- CDC, Guideline for the Prevention of Surgical Site Infection, 2017.
- WHO, Global Guidelines for the Prevention of Surgical Site Infection (2016/2018).
- AORN, Guidelines for Perioperative Practice; SHEA/IDSA/APIC, Strategies to Prevent SSI (2022 Update).
- CDC NHSN, Surgical Site Infection Event surveillance definitions. Related: POL-RM-013, POL-IC-020.